

DISCHARGE SUMMARY

PATIENT INFORMATION

Name: Patient 42

Age: 74 years old | Sex: Female

MRN: 000-TEST-42

Admission Date: [Current Date - 3 days] | Discharge Date: [Current Date]

Attending Physician: Dr. M. Chen, General Surgery

PRIMARY DIAGNOSIS & SURGICAL PROCEDURE

Acute gangrenous cholecystitis s/p laparoscopic cholecystectomy with intraoperative conversion to open procedure due to dense adhesions and unclear anatomy. Patient presented to Metro Medical Center with RUQ pain, fever (101.8°F), and elevated WBC (14.2 K/uL). HIDA scan demonstrated cystic duct obstruction with acute inflammatory changes.

SECONDARY DIAGNOSES

- Atrial fibrillation (paroxysmal, rate-controlled on current regimen)
- Obesity (BMI 34.2)
- Hypertension
- Type 2 Diabetes Mellitus
- Hyperlipidemia

HOSPITAL COURSE

Patient admitted POD 0 after presenting via ED with acute abdomen. CT abdomen/pelvis showed gallbladder wall thickening (6mm) with pericholecystic fluid and cholelithiasis. Surgical intervention scheduled emergently. Procedure initiated laparoscopically via three trocars (10mm umbilical, 5mm epigastric, 5mm midaxillary). Due to severe adhesive disease

from prior appendectomy (history per patient), conversion to open supraumbilical incision was required at 35 minutes. Gallbladder removed intact; CBD patent on intraoperative cholangiogram. Blood loss minimal (~75 mL). Patient tolerated procedure well.

Postoperative course complicated by atrial fibrillation with RVR (HR 118–132) on POD 1; rate controlled with IV diltiazem and reinitiation of home metoprolol. Trocar sites healing without erythema or drainage. Pain controlled on scheduled acetaminophen with PRN oxycodone. Ambulating independently on POD 2. Tolerating PO intake by discharge (advanced from clear liquids → full diet over 48 hours without nausea/vomiting).

DISCHARGE MEDICATIONS

| Medication | Dose | Frequency | Duration |

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| Metoprolol tartrate | 50 mg | BID | Ongoing |

| Diltiazem ER | 180 mg | q.d. | Ongoing |

| Metformin | 500 mg | BID with meals | Ongoing |

| Atorvastatin | 40 mg | q.d. at bedtime | Ongoing |

| Acetaminophen | 650 mg | TID scheduled | 2 weeks |

| Oxycodone 5 mg | 1–2 tablets | q.4–6h PRN pain | 1 week |

| Docusate sodium | 100 mg | BID | 2 weeks |

ACTIVITY RESTRICTIONS & WOUND CARE

- NO lifting >10 lbs × 6 weeks

- Avoid strenuous activity, heavy labor

- Supraumbilical incision: Keep dry × 5 days; may shower thereafter with care to keep incision dry

- Trocar sites: May be exposed to air; observe for signs of infection

- Resume ADLs as tolerated; sleep supine or on left side for comfort

DIET

- Clear liquids × 24h post-discharge → advance to regular diet as tolerated
- High-fiber diet preferred after acute phase resolves (week 3–4) to promote GI motility
- Avoid fatty foods × 4 weeks (may precipitate symptoms)
- No